



Policy

Identifying people of Aboriginal or Torres Strait Islander origin

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Admin and Clerical

Introduction

Hospitals have a responsibility to collect data from all our patients to better understand the health needs of all Australians.

The collection of every patient's Aboriginal status is necessary for the government and other services to:

- plan and deliver appropriate health services for all Australians;
- measure the impact of services on particular groups; and
- monitor trends and changes in the health and wellbeing of Australians over time.

The most accurate and reliable way to collect Aboriginal status is to have patients answer the standard identification question for themselves.

One simple question could help 'Close the Gap'.

Definitions

Aboriginal	This policy uses the term Aboriginal. Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. Aboriginal and Torres Strait Islander may be referred to in the national context and Indigenous may be referred to in the international context. No disrespect is intended to Torres Strait Islander colleagues and community.
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Risk Statement

Non-compliance with this guidance will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other: NSQHS Standard 5.8	☒		

1.0 What is the question?

The Australian Institute of Health and Welfare (AIHW) recommends the exact wording for this question as: **"Are you of Aboriginal or Torres Strait Islander origin?"**

This wording is consistent across Australia and has been the same since the 1981 Australian Bureau of Statistics census.

If a patient is unable to respond, staff can instead ask the following question of the carer or responsible accompanying adult: **“Is (person’s name) of Aboriginal or Torres Strait Islander origin?”**

It is important that the question is asked exactly as it is worded. Changing the question even slightly has the potential to alter the question’s meaning, and this may in turn influence the patient’s response.

2.0 Who must ask the question?

Staff responsible for registering a patient must ask the standard question. Patients may also be asked via forms or questionnaires.

3.0 Who must be asked the question?

Staff must ask the question of all patients irrespective of appearance or country of birth. Choosing not to ask the question, based on the above, could be regarded as an act of discrimination - All patients have the right to self-report their Aboriginal status, rather than have their status assumed and recorded on their behalf.

If the patient is unable to respond

When the patient is unable to respond to the standard question, in the first instance the staff member should ask the patient’s carer or responsible accompanying adult. The response provided by this person should be verified with the patient when they have recovered sufficiently to be able to answer the question for themselves.

If the person accompanying the patient does not know whether the patient is Aboriginal and/or Torres Strait Islander, the patient should be asked the question directly when they are capable of responding. If the patient does not recover sufficiently to provide this information, the answer to the standard question on Aboriginality should be recorded as ‘Not Stated’.

If the patient does not speak English

If the patient does not speak English but is accompanied by someone who can interpret for them, it is recommended that the person accompanying them is asked to translate the question and their response.

If there is no-one with the patient who can speak English, it is recommended that an interpreter be called to assist.

4.0 When to ask the question?

The question must be asked at the point of intake, when a patient’s other details, such as name, date of birth, and address are being collected or checked.

Common areas where staff must ask the question include:

- Emergency Department
- Admissions
- Outpatients (Initial registration for a single treatment or course of treatment)

All patients should have the opportunity to confirm or update any previously recorded personal information, including confirmation or alteration of a record that they are Aboriginal and/or Torres Strait Islander.

Patients should be afforded the option to update or change their response:

- Every 6 months (for a prolonged course of treatment)
- If there is a noted discrepancy between different data sources

5.0 How to record the answer?

Patients have a right to decide how they will answer the question, and staff should record the response that the patient provides.

Staff should provide the following response options:

- Aboriginal and TSI;
- Aboriginal not TSI;
- Neither; or
- TSI not Aboriginal.

A missing response on a returned form should be followed up immediately and should not be automatically regarded as a decision to decline to respond.

If a patient indicates that they do not wish to answer this question, the response is recorded as 'Not Stated' for no further follow-up.

If a patient objects to the question or says they don't want to answer, they should be assured that their standard of care will not be affected if they choose not to answer.

Staff may provide information such as this fact sheet produced by the AIHW: ['Are you of Aboriginal or Torres Strait Islander Origin?'](#)

While staff have a duty of care to ask the question, staff are not obliged to convince an unwilling patient to respond - all patients have the right to refuse to respond.

6.0 What are the benefits of asking the question?

Benefits for the health service organisation include:

- Improved standards, more consistent practice, improved accuracy in recording status, and improvements in meeting the needs of Aboriginal and Torres Strait Islander people;
- Accurate data on Aboriginal and Torres Strait Islander people to inform policy, service planning and development;
- Accurate monitoring of disparities in health care and outcomes used to inform actions to reduce these disparities; and
- Accuracy of financial remuneration in providing services to Aboriginal and Torres Strait Islander people through activity-based funding and service loadings.

Benefits for the Aboriginal and Torres Strait Islander community include:

- Safe, high-quality and culturally appropriate clinical care;
- Understanding that self-identification is important to enable access to services including Aboriginal Hospital Liaison Officers;
- Upholding of the rights of all Aboriginal and Torres Strait Islander people; and
- Improved quality of services available to individual patients and their families by informing care requirements, including population-specific improvements in clinical decision-making.

7.0 Where to find more information?

A brief 5-minute training course is available on MyLearning. The training will ensure all staff responsible for collecting information are able to demonstrate:

- An ability to ask the standard question 'Are you of Aboriginal or Torres Strait islander origin?' correctly, and to correctly record responses on paper forms and / or computer systems.

- An ability to clearly explain to patients the reason for collecting this information.
- An understanding of why it is important to collect and record information on whether all patients are Aboriginal and/or Torres Strait Islander.
- An understanding of why it is important to collect this information correctly and consistently, using the standard question.
- An understanding of the voluntary nature of self-reporting a patient's Aboriginality, and of a patient's right to decline to answer this question or to change the information recorded.
- Knowledge of available information and services for Aboriginal and Torres Strait Islander patients, and ability to convey this to patients as required.
- Knowledge of and ability to conduct follow-up procedures for obtaining missing information, including whether a patient is Aboriginal and/or Torres Strait Islander.

Staff can also access information from the Australian Commission on Safety and Quality in Health Care, specifically National Standard [Action 5.8: Identifying people of Aboriginal and/or Torres Strait Islander origin](#).

8.0 How to update status

Should a patient identify as Aboriginal at point of intake, however subsequent interactions reveal this is incorrect (such as assessment by Aboriginal Hospital Liaison Officers), staff should notify the ED / Ward / Outpatient clinic clerk of the error.

The clerk will then confirm the change in demographic status with the patient or their next of kin and directly update webPAS or notify the Patient Master Index team to make the change.

Compliance

Annual, random audits will be completed to ensure staff involved in the collection of patient information are asking the question in line with this policy.

Outcomes (and actions) will be tabled at committees including but not limited to:

- Aboriginal Health and Wellbeing Committee
- Outpatient Committee
- Corporate Governance Committee

Related Documents

[National best practice guidelines for collecting Indigenous status in health data sets](#)

[WA Health Mandatory Policy MP 0190/25 Aboriginal Data Governance Policy](#)

Authorisation & Version Control

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Applicable Standards	NSQHS Standards				
	Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 5: Comprehensive Care 				
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